FEVER WITH RASH



Mentor
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Presentation by
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PATIENT PARTICULARS

- Age : 4 years
- Sex : Female
- Residence : Yeshwantpur, Bangalore
- Informant : Mother (reliable)
- Socioeconomic status : Lower Middle (Modified Kuppuswamy Classification)
- Date of Admission : 27.9.2021
- Date of Examination : 29.9.2021

CHIEF COMPLAINTS

- Fever from 5days
- Vomitting from 4 days
- Cough from 3 days
- Rash from 3 days

HISTORY OF PRESENTING ILLNESS

- Child was apparently alright 5 days ago when she developed fever. Initially it was low grade, without chills and rigors. It was relived on taking medication (paracetamol syrup) for 4-5 hrs after which the temperature would rise again. Over the period of 5 days, the fever progressed to high grade with chills and rigors. Child was tired during the inter febrile period, and did not consume the regular amount of food.
- On day 2, she had 4-5 episodes of vomiting, non projectile, contained food particles, was non bile stained, non blood tinged, which would happen few hours after having her meals.
- She developed cough the next day, which was non productive, had progressed from few bouts to 8-10 bouts each cough, aggravated at night. There was no post tussle vomitting.

HISTORY OF PRESENTING ILLNESS

- She also developed rash, which was insidious in onset. Her mother first noticed the rash on her face, which then progressed to her neck, abdomen, upper limbs and legs.
- The rash was maculo papular, pinkish in color, initially discrete and then had become confluent. It was associated with itching. The fever had progressed to having chills and rigors after the onset of the rash. Rash did not involve palms and soles.
- Fever was not associated with
- Throat pain, ear pain / discharge , runny nose.
- Change in voice, hurried breathing, noisy breathing
- passage of loose stools, pain abdomen
- headache, blurring of vision, altered sensorium
- Burning micturition, decreased urine output

HISTORY OF PRESENTING ILLNESS

- No history of
- Join pain, bleeding manifestation.
- Travel, exposure to pets/ animals.
- No history of similar complaints in siblings, neighbors.
- No history of contact with Covid patient.
- No History of recent immunization.

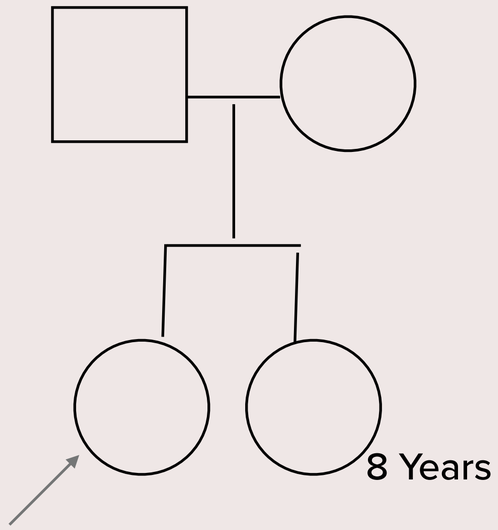
PAST HISTORY

- Wellness visits : For immunization
- Illness visits : No history of previous hospitalization

FAMILY HISTORY

- Non consanguineous marriage
- No similar complaints

  in sibling



BIRTH HISTORY AND NEONATAL HISTORY

- Full term by LSCS. Birth weight 2.6kg.
- Cried at birth
- No history of NICU admission.
- Neonatal period was uneventful.

IMMUNIZATION HISTORY

- Immunized unto date as per National Immunization Schedule. Last being at 2 years of age.

DEVELOPMENTAL HISTORY

- Developmental milestones are appropriate for age in all domains
- Gross motor: Walks with alternate foot going down the stairs.
- Fine motor: can draw squares
- Social and adaptive : Plays in a group. Can go to toilet on her own.
- Language : sings songs, tells stories.

DIET HISTORY

- Exclusively breastfeed for 6 months after birth and breast feeding was contained unto 2 years of age.
- Complementary feeding was started after 6 months of age.
- 24 hrs diet history
- Total calories consumed : 1030kcal
- Requirement : 1350 kcal
- Total protein consumed : 20g
- Required protein : 24g

Food consumed	CALORIES	PROTEINS
BREAKFAST : 2 IDLIS	150	4g
Kichdi : Lunch	180	6g
Kichdi : Dinner	180	6g
Snacks Banana	116	1.2
2 pieces of plane cake	114*2	3g

SUMMARY

- 4 yrs old girl child, 2nd born to non consanguineous marriage, immunized unto date, with developmental milestones appropriate for age,
- Presented with fever since 5 days initially low grade which progressed to high grade after appearance of rash, maculopapular, centrally distributed, initially discrete becoming confluent, associated with decreased appetite and tiredness, 4-5 episodes of vomiting from 4 days and non productive cough from 3 days.

- The above history is suggestive of fever with rash due to an infectious etiology.

GENERAL PHYSICAL EXAMINATION

- Child was conscious, alert and toxic looking.
- VITALS:
- Temperature : 98.4 F (axillary)
- Pulse : 102 beats per minute, regular in rhythm, normal in volume and character, no radio radial or radio femoral delay.
- Respiratory rate : 18 beats per minute
- BP : 100/60 mm Hg, Right arm sitting position

ANTHROPOMETRY

Particulars	Observed	EXPECTED	Percentile	A
Weight	15kg	16kg	3rd - 50th centile	Appropriate for Age
Height	107cm	101cm	50- 97th centile	Appropriate for age
MUAC	16cm	>13.5cm		Appropriate for age
Head Circumference	48 cm	50cm	15-50th centile	Appropriate for age

HEAD TO TOE EXAMINATION

- Head : hair appears normal
- Eyes : Conjuctival congestion.
- Nose: External nose normal, no deviation of nasal septum, no discharge
- Ears : B/L intact tympanic membrane, no discharge.
- Oral cavity : 2 whitish spots, 2-3mm in size, opposite to lower second molar, on both sides.
- Tonge : Appears normal.
- Sides of lips appear dry.
- Skin : Pink, confluent maculopapular rash, on face, neck, abdomen, upper and lower limbs. Less marked on face and neck.
- Neck : No enlarged lymph nodes.
- No Pallor, Icterus, Cyanosis, Clubbing, Lmyphadenopathy, Pedal edema.

SYSTEMIC EXAMINATION

- Respiratory system :
- Upper respiratory tract : Normal
- Inspection : Chest shape normal , B/L symmetrical
- Trachea central
- No use of accessory muscles of respiration
- No scars/ sinuses
- Palpation : inspectors findings confirmed. Trachea central, chest expansion B/L symmetrical, Vocal Fremitus equal on both the sides,
- Percussion : Resonant note in B/L supraclavicular, infraclavicular, mammary, axillary , infra axially, supraclavicular , inter and infra clacivular.
- Ausculatation : B/L Normal vesicular breath sounds. No added sounds.

SYSTEMIC EXAMINATION

- Per Abdomen (supine position)
- Inspection : appears normal.

Corresponding quadrants move equally with respiration.

Umbilicus is central.

Rash, maculopapular

No scars or sinuses

Hernial orifices intact.

Palpation : No local rise in temperature

No tenderness.

Soft and non tender.

No hepato splenomegaly

Auscultation : Normal Bowel Sounds heard.

SYSTEMIC EXAMINATION

- Nervous system : No neurological deficits.
- CVS : s1 s2 heard. No murmurs.

PROVISIONAL DIAGNOSIS

- Case of fever with rash,
- Could be :
- Measles
- Dengue
- Rickettsia
- Malaria
- MISC